

IBD — Crohn's vs Ulcerative Colitis



1. CROHN banner / skip-lesion fires

WHAT YOU SEE

Left half flagged 'CROHN' with scattered separate campfires.

WHAT IT ENCODES

Crohn's disease = SKIP LESIONS — patchy inflammation with normal mucosa between affected segments (the separated fires). Can affect anywhere mouth-to-anus. Mnemonic: granuloma + skip + transmural = Crohn's.

2. Ileum 90% (burning tent)

WHAT YOU SEE

A burning tent labeled 'ILEUM 90%'.

WHAT IT ENCODES

Terminal ileum is involved in ~90% of Crohn's cases (ileitis/ileocolitis), explaining B12 and bile-acid malabsorption and calcium-oxalate stones. Transmural inflammation here drives strictures and fistulae.

3. Rectum spared (cold corner)

WHAT YOU SEE

Lower-left corner labeled 'RECTUM SPARED', no fire.

WHAT IT ENCODES

In Crohn's the rectum is typically SPARED — contrast with UC where the rectum is ALWAYS involved. Rectal sparing plus skip lesions strongly favors Crohn's on colonoscopy.

4. Fistula (burrows/barrel)

WHAT YOU SEE

Labels 'FISTULA' near a barrel and burrowing tunnels at the bottom.

WHAT IT ENCODES

Transmural inflammation in Crohn's creates FISTULAE (entero-enteric, perianal, enterovesical) and abscesses — a hallmark absent in mucosal-only UC. Perianal fistulizing disease especially favors Crohn's; infliximab is best for fistulizing CD.

5. ASCA knight

WHAT YOU SEE

Kneeling knight labeled 'ASCA'.

WHAT IT ENCODES

ASCA (anti-Saccharomyces cerevisiae antibody) is positive in Crohn's. Contrast: p-ANCA positive in UC. The pair (ASCA+ Crohn / p-ANCA+ UC) is a high-yield serologic discriminator.

6. Creeping fat (fox)

WHAT YOU SEE

A fox/creature creeping near the tables (creeping fat cue).

WHAT IT ENCODES

Creeping fat = mesenteric fat wrapping the bowel, a Crohn's-specific transmural finding. Along with cobblestoning mucosa it signals deep, full-thickness inflammation.

7. Granuloma monks (gold star)

WHAT YOU SEE

Hooded monks huddled with a gold star, labeled 'GRANULOMA'.

WHAT IT ENCODES

Non-caseating granulomas are the histologic signature of Crohn's (present in a minority of biopsies but specific). Caseating granulomas instead suggest TB — Crohn's granulomas are NON-caseating.

8. GI lymphoma (red X)

WHAT YOU SEE

Two figures carrying a stretcher labeled 'GI LYMPHOMA', red X.

WHAT IT ENCODES

GI lymphoma is a distractor differential — terminal-ileal mass/wall thickening can mimic Crohn's but is not IBD. Note immunomodulators/anti-TNF slightly raise lymphoma risk, but lymphoma itself is not the diagnosis here.

BOARD TRAP

"This Crohn's-like ileal disease is GI lymphoma" → wrong; it is a mimicking distractor, not the IBD diagnosis.

9. Indeterminate colitis (red X)

WHAT YOU SEE

Hooded figure labeled 'INDETERMINATE', red X.

WHAT IT ENCODES

Indeterminate colitis is a label only when features overlap and Crohn vs UC can't be separated — a distractor when the classic discriminators (skip vs continuous, granuloma vs crypt abscess) are actually present.

BOARD TRAP

"Classic granuloma + skip lesions = indeterminate colitis" → wrong; those define Crohn's, not indeterminate.

10. UC banner / red continuous flood

WHAT YOU SEE

Right half flagged 'UC' with a continuous red flooded floor.

WHAT IT ENCODES

Ulcerative colitis = CONTINUOUS inflammation extending proximally from the rectum (the unbroken red flood). Limited to mucosa/submucosa. Mnemonic: continuous + rectum + mucosal = UC.

11. Rectum always involved

WHAT YOU SEE

Doorway label 'RECTUM ALWAYS'.

WHAT IT ENCODES

UC ALWAYS involves the rectum and spreads continuously proximally (proctitis → left-sided → pancolitis). Constant rectal involvement plus continuity distinguishes UC from rectum-sparing, skip-lesion Crohn's.

12. Pseudopolyps

WHAT YOU SEE

Green island-like projections labeled 'PSEUDOPOLYPS'.

WHAT IT ENCODES

Pseudopolyps are islands of regenerating mucosa between ulcers, typical of UC. With lead-pipe (featureless, haustra-lost) colon they reflect chronic continuous mucosal damage. Pseudopolyps are NOT malignant.

13. p-ANCA queen

WHAT YOU SEE

Purple-robed queen labeled 'p-ANCA'.

WHAT IT ENCODES

p-ANCA is positive in UC (and associated PSC). Pairs against ASCA+ for Crohn's. Useful serologic discriminator, though imperfect sensitivity.

14. Backwash ileitis (pipe)

WHAT YOU SEE

Large pipe at right with 'BACKWASH' label and red spilling.

WHAT IT ENCODES

Backwash ileitis = mild inflammation of the terminal ileum in pancolitis UC from reflux of cecal contents through an incompetent ileocecal valve — a UC phenomenon, not true Crohn's ileitis. Crypt abscesses are the UC histology.
